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Clinical Presentation and Examination

Clinical presentation

- Episodic headache lasting from 30 minutes to 7 days
- Not associated with nausea or vomiting
- Photophobia or phonophobia may be present (not both)
- At least two of:
 - Bilateral location
 - Pressing, tightening or non-pulsating quality
 - Mild or moderate intensity
 - Not aggravated by routine physical exercise e.g. walking
- May be triggered by stress, missing meals, sleep deprivation and lack of physical activity
- Not caused by another condition such as pyrexia, medication overuse or caffeine withdrawal
- Can occur in combination with migraine and secondary headache triggers
- Assess for symptoms of depression and impact on quality of life
- *Disabling tension-type headache is rare and may reflect another diagnosis*

Examination

- Examination should include vital signs, neurological examination, neck examination, extracranial structures (such as TMJ) and fundoscopy
- Normal neurological examination

ICHD categorises tension-type headache according to attack frequency:

Infrequent episodic – at least 10 episodes of headache occurring on less than 1 day per month on average (<12 days per year)

Frequent episodic – at least 10 episodes of headache occurring on fewer than 15 days per month on average, for more than 3 months

Chronic – evolves from frequent episodic tension-type headache, headache occurring on 15 days or more per month on average, for more than 3 months and:

- Lasting hours to days, or unremitting.
- At least two of bilateral location; pressing or tightening (non-pulsating) quality; mild or moderate intensity; and not aggravated by routine physical activity such as walking or climbing stairs.
- Neither moderate or severe nausea nor vomiting *and* no more than one of photophobia, phonophobia, or mild nausea.
- Not better accounted for by another cause, such as medication overuse.

Any Red Flags

- Progressive or persistent headache, or headache that has changed dramatically
- 'Thunderclap headache'
- Associated features including: fever, impaired consciousness, seizure, neck pain/stiffness, photophobia, papilloedema, new onset neurological deficit, change in personality, cognitive dysfunction, altered consciousness, atypical aura (>60 mins, motor weakness/double vision/impaired balance/visual symptoms affecting one eye, 1st time aura in a person taking the COCP), vomiting, dizziness, visual disturbance
- Contacts with similar symptoms e.g. carbon monoxide poisoning
- Precipitating factors: head trauma, triggered by Valsalva manoeuvre/sneeze/cough, worse on standing, lying down or exercise
- Comorbidities such as those who are immunocompromised or have current or past malignancy
- Current or recent pregnancy
- New onset headache in a person > 50 years

General Management

Provide information and support using patient information sources

Advise on the risk of medication overuse headache

Consider a headache diary to record frequency and duration of headaches and to monitor effectiveness of treatment

Assess for triggers such as stress, mood disorders, poor posture, neck pain, diet and sleep disorders and manage appropriately

If co-existing medication over use headache or migraine – manage appropriately as per NICE guidance (CG150)

If there is diagnostic uncertainty or atypical symptoms consider advice and guidance to neurology or arranging referral to neurology

Do not routinely refer people with tension-type headache for neuroimaging for reassurance purposes only

Infrequent Episodic tension type headache

Alongside general management

- Usually self-limiting and Advise to take OTC simple analgesia (paracetamol, aspirin or NSAIDs) as soon as possible after onset of an acute attack
- Do not treat acute TH with opioids
- Further prescribing guidance can be found on NICE guidelines and the BNF

Frequent episodic or chronic tension type headache

- Advise avoid frequent and excessive use of acute analgesia due to risk of medication overuse headache
- Advise a course of 6-10 sessions of acupuncture treatment (Acupuncture is not routinely available on the NHS but if pursued privately recommendations are that it should be a course of 6-10 sessions)
- Amitriptyline 10mg nocte increasing in increments of 10-25mg every 1-2 weeks, up to a maximum dose of 150mg ('off label')
 - Should be used instead of acute treatment for attacks
 - Suggest trial of 2-3 months before stopping if is ineffective. If good response after 6 months – consider gradually reducing
- Consider CBT, regular exercise, physiotherapy and/or relaxation techniques – may be helpful in some people (NICE CKS and BASH)
- Headache calendar can be used to monitor symptoms and response to treatment
- Further prescribing guidance can be found on NICE guidelines and the BNF

Symptoms unresolved.

If symptoms persist:

- Check concordance with drug treatment, check treatment has been offered at the maximum tolerated dose and for an appropriate duration, review frequency of acute medications and reassess for an underlying or alternative diagnosis
- If symptoms persist despite optimal management in primary care – considering seeking specialist advice through advice and guidance

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- If symptoms persist despite optimal management in primary care – seek specialist advice or consider arranging a referral to neurology



Information for patients

Tension-type Headache from Patient Info UK: <https://patient.info/health/tension-type-headache-leaflet>



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References

<https://www.nice.org.uk/Guidance/CG150>

<https://cks.nice.org.uk/topics/headache-assessment/>

<https://cks.nice.org.uk/topics/headache-tension-type/>

https://bash.org.uk/wp-content/uploads/2023/02/01_BASHNationalHeadache_Management_SystemforAdults_2019_guideline_versi-1.pdf